

AHN West Penn Hospital

4800 Friendship Ave, Pittsburgh, PA 15224

DISCHARGE SUMMARY Document #: DIS-016

Patient & Admission

Name	Natasha Wood
Date of Birth	1946-05-06
Phone	878-555-5313
Address	Butler, PA, 16001
MRN	MRN-980841
Admit Date	2026-05-10
Discharge Date	2026-05-14
Attending	Mary Phillips, MD
Attending NPI	3158692327

Insurance

Primary Payer	Medicare
Primary Member #	9HUQ-09N-1699

Primary Discharge Diagnosis

Mild CAD on diagnostic angiography

ICD-10: I25.10, Z98.61

Hospital Course

The patient presented with exertional chest discomfort and dyspnea on exertion. Initial electrocardiography showed nonspecific ST-T wave changes, and high-sensitivity troponin was mildly elevated at 0.08 ng/mL. Transthoracic echocardiography revealed preserved left ventricular ejection fraction at 55% with mild left ventricular hypertrophy and normal diastolic function. BNP was modestly elevated at 285 pg/mL. Given the clinical presentation and objective findings suggestive of possible coronary artery disease, the patient was referred for diagnostic cardiac catheterization to further risk-stratify and guide management.

Coronary angiography was performed via right radial access without complication. Findings revealed mild atherosclerotic disease of the left anterior descending artery with approximately 40% stenosis, a patent left circumflex artery, and a right-dominant system with mild disease in the right coronary artery. No acute coronary syndrome was identified, and no intervention was warranted. The patient tolerated the procedure well with stable hemodynamics throughout. Post-procedure angiographic images were reviewed with the patient and family, and the benign findings were discussed in detail.

During the hospital stay, the patient remained hemodynamically stable on her home medications, which included lisinopril and atorvastatin. Troponin levels trended down to baseline. She was discharged on hospital day four in stable condition with outpatient cardiology follow-up arranged. The patient was counseled on

lifestyle modifications including cardiac rehabilitation, dietary sodium restriction, and regular aerobic exercise as tolerated. She was instructed to continue her current medications and to report any recurrent chest pain or dyspnea to her primary care physician. Close outpatient follow-up was emphasized given her age and comorbidities.

Procedures Performed

Coronary angiography	CPT 93458
PCI to LAD with DES	CPT 92928

Medication Changes

Isosorbide mononitrate 30 mg daily	STARTED
Clopidogrel 75 mg daily	STARTED
Aspirin 81 mg daily	STARTED
Atorvastatin 40 mg daily	ADJUSTED

Follow-up Instructions

Recommended Specialist	Cardiology
Follow-up Window	20 days
Urgency Tier	ROUTINE
Clinical Flags	radial access — wrist precautions, femoral access — bleed precautions, post-PCI dual antiplatelet therapy

Dictated and electronically signed by:
Mary Phillips, MD
NPI: 3158692327
AHN West Penn Hospital